

CONSENT TO USE ELECTRONIC SIGNATURES

Client Name: _____

MRN: _____

DOB: _____

Dear Client,

We are excited to announce that S&B Behavioral Health Care LLC is embracing the digital age with the introduction of our new electronic signature system. This innovative solution is designed to streamline our processes while reducing paper use, ensuring that agreements and documents can be executed swiftly and securely.

Please review the following statements detailing the terms of the ESIGN Act and how it governs our electronic transactions. Your electronic signature below will signify your agreement to these terms and your willingness to move forward with us in this digital endeavor.

The Electronic Signatures in Global and National Commerce Act (ESIGN Act) is a United States federal law that was enacted on June 30, 2000. The ESIGN Act ensures the validity and legal effect of contracts entered into electronically, making electronic signatures and records as legally binding as their traditional paper counterparts. Key provisions of the ESIGN Act include: a general rule of validity for electronic records and signatures, specific exceptions, applicability to government agencies, and the establishment of transferable records for electronic transactions. The Act also outlines consumer rights, including the requirement for clear and conspicuous consent to use electronic records and the right to withdraw consent with certain conditions. It's important to note that the ESIGN Act does not require individuals to use electronic signatures; it simply provides the legal framework for their use if both parties agree to proceed electronically.



CONSENT TO USE ELECTRONIC SIGNATURES

Please read the following statements:

1. Intent to Sign: By using our electronic signature system, you agree to have your electronic signature serve as the legal equivalent of your manual signature.
2. Consent to Electronic Delivery: You agree that we may provide all communications, agreements, and receipts in electronic format.
3. Withdrawal of Consent: You have the right to withdraw your consent to receive/submit electronic signatures, but if you do, we may choose not to do business with you.
4. Paper Copies: You may request paper copies of electronic records, but we reserve the right to charge a fee for such copies.
5. Updating Contact Information: It is your responsibility to keep your email address current so that we can communicate with you electronically.

Please sign below to indicate that you have read and agree to these terms.

Client Signature

Date



Permission is hereby given to **S&B Behavioral Health Care LLC** to release, for professional use only, information to include medical, psychiatric, psychological and substance abuse evaluations, services and treatment contained in the clinical record of:

Client Name: _____

MRN: _____

DOB: _____

This release complies with federal regulations (*42 CFR Part 2*) and all applicable state and local laws, rules, and regulations. Information released may not be further disclosed without permission from the client or representative. Information released may not be used to criminally investigate or prosecute a substance abuse client. The client or representative has been given the opportunity to discuss the possible benefits and disadvantages of releasing this information. The authorization is freely given, and the provision of services is not dependent on the client's or representative's decision regarding release.

This release will be terminated one year from the date of signature unless otherwise noted. A client or representative may withdraw this permission at any time in writing, however, action taken prior to cancellation cannot be revoked.

Name of agency releasing information: **S&B Behavioral Health Care**



Emergency Contact

Name: _____

Address: _____

Cell Phone Number: _____

Home Phone Number: _____

Relationship: _____

Purpose of Release

Emergency Contact Information to be released: *(fill up all that apply)*

Medical Information: _____

Client Location _____

Client Signature

Date



REQUEST FOR INTERPRETATION

Client Name: _____

MRN: _____

DOB: _____

Maryland (COMAR) regulations require SUD providers to provide interpretation services to help you speak with and understand your healthcare professional and office staff.

Please fill out completely and check all that apply

- ☐ I need an interpreter: Yes, I need an interpreter to speak to my healthcare professional and/or office staff. I prefer to speak the following language: _____
- ☐ I need an interpreter, but I prefer to use my family, friend, and/or healthcare professional office staff as an interpreter.
- ☐ I do not need an interpreter: I do not need or want an interpreter.

By signing below, I confirm that I have been offered an interpreter to help me speak with and understand my healthcare professional and/or office staff.

Client Signature

Date



Client Name: _____

MRN: _____

DOB: _____

Consent to Treatment

I consent to care and treatment by **S&B Behavioral Health Care LLC**, its affiliates, and its employees. I also consent to treatment and care by physicians, behavioral health providers, and healthcare professionals who are not employees or agents of **S&B Behavioral Health Care**, but are authorized by **S&B Behavioral Health Care** to provide treatment and care to me. I understand that my care team at **S&B Behavioral Health Care** may include interns/students or other trainees, under the supervision of the certified/licensed employees.

I understand that my treatment and care will include substance use disorder treatment, including but not limited to group, individual, and family counseling and recreational/therapeutic activities. I understand that **S&B Behavioral Health Care** is an integrated behavioral health system with multiple treatment programs. By signing this form, I understand I am consenting to treatment in any program recommended by my **S&B Behavioral Health Care** provider.

I acknowledge that the specific benefits and risks of treatment, factors influencing the likelihood of success, treatment alternatives, and my right to refuse treatment services have been explained to me. I acknowledge that sufficient information and explanation concerning the nature and purpose of **S&B Behavioral Health Care's** treatment programs, the procedures, and methods of treatment have been explained to me in order for me to make an informed judgment about my treatment. I am aware that substance use disorder treatment is not an exact science, and I acknowledge that no one has made any guarantees about the results of my treatment.

Consent to Telehealth Services

I consent to receive treatment and services through telehealth, including through interactive video or audio platforms. I understand that some communication platforms may not be secure, but are allowed under emergency rules. I consent to receiving services via telehealth which may have some risks, including unsecure or unencrypted transmission, audio and video interruptions, access by unauthorized persons, or unexpected disruptions or distortions from technical failures. Although it is unlikely, I understand my protected health information may be breached if someone tampers with the technology. This consent will remain in effect until revoked. I understand that I



have the right to revoke my consent at any time by notifying my provider. I understand any technology or data charges incurred while downloading or using telehealth services on any of my personal devices will be my responsibility. I understand that I may not record any telehealth services without written permission.

Voluntary Nature of Program

I voluntarily consent to treatment at **S&B Behavioral Health Care** and that the success of my treatment rests in my willingness to cooperate with the treatment process. I acknowledge that, unless mandated by court officials, I may leave treatment at any time. I hereby release **S&B Behavioral Health Care** from all responsibility for any acts or consequences, medical or otherwise, which may result from my leaving treatment without authorization.

S&B Behavioral Health Care Rules

Upon admission, I received a client orientation handbook. I agree to cooperate and abide by all **S&B Behavioral Health Care** rules contained therein. I understand that **S&B Behavioral Health Care** may terminate services if I fail to comply with all rules in the handbook. I understand that, upon admission to **S&B Behavioral Health Care** and from time-to-time thereafter, my person and property will be searched for contraband. While in treatment, all alcohol, drugs, medications without appropriate prescription, or drug paraphernalia in my possession and/or brought onto **S&B Behavioral Health Care** property will be destroyed.

Client Rights and Confidentiality

I have had my client rights and responsibilities explained to me. I have been given a copy of and clearly understand the statement that describes the rights I have in treatment. I understand that my treatment at **S&B Behavioral Health Care** is subject to strict federal and state confidentiality laws.

During the program, such as in group sessions, I may learn confidential information about other clients in treatment. I agree to keep all information about other clients, including their participation in the program, in the strictest confidence.



Personal Property and S&B Behavioral Health Care Property

I understand that **S&B Behavioral Health Care** is not responsible for personal property that is lost, stolen, damaged, or left behind. I accept responsibility for any damage or destruction that may occur to **S&B Behavioral Health Care** property as a result of my behavior while I am a **S&B Behavioral Health Care** client.

Release of Liability

I understand that part of my treatment may involve activities including, but not limited to, the participation in recreational/therapeutic activities and transportation by **S&B Behavioral Health Care** ("Activities"). These Activities involve the risk of accident, personal injury, and property damage. I understand that I can refuse to participate in these Activities. I agree for myself, my heirs, assigns, or representatives to waive, release, and forever discharge **S&B Behavioral Health Care**, its affiliates, and its employees from and against any and all claims, liabilities, and causes of action, whether foreseeable or unforeseeable should any incident occur involving personal injury or property damage during my participation in treatment,

Consent to Videotape/Audiotape

I understand that some **S&B Behavioral Health Care** facilities may employ video surveillance equipment for security monitoring purposes. I further understand that it is possible that my image will be captured by that equipment and that such images are protected by state and federal confidentiality laws. I also understand that some of my treatment sessions may be audio taped for internal performance improvement and quality assurance with my signed consent to do so on a separate form outside of this one. My signature on this consent for treatment form does not give permission for **S&B Behavioral Health Care** to audio tape my treatment sessions. I understand that I must sign a separate, designated consent form in regards to my sessions being audio taped.



Consent for Electronic Communication

I give my consent for **S&B Behavioral Health Care** to communicate with me electronically via email, voicemail, or text message. I understand and acknowledge that there are risks inherent in the electronic transmission of unencrypted information over the internet or cellular networks and that such communications may be lost, delayed, intercepted, corrupted, or otherwise not delivered. I understand I may revoke this consent in writing to the Compliance Department at **S&B Behavioral Health Care** at any time. In the case of a crisis or emergency, email and text message communication is not an appropriate method of reaching **S&B Behavioral Health Care** staff.

Please select all that apply.

I consent to electronic communication: ☐ Yes ☐ No

Appointment Communication/Reminder Preference:

☐ Phone Call/Voicemail ☐ Email ☐ Text Message

I certify that I have read the above form, that I understand its contents, and that I have asked all questions I have about this consent for treatment form.

Client Signature

Date



CONSENT FOR SERVICE FEE AND AGREEMENT

Client Name: _____

MRN: _____

DOB: _____

I understand that I am entering the substance abuse treatment program of **S&B Behavioral Health Care LLC** and agree with full knowledge that I will attend all sessions that have been outlined in my treatment plan. I also understand that missing two consecutive appointments, without prior approval/notice, will be grounds for dismissal from this program, as will not having face-to-face contact with a member of the program staff for a period of 30 days. All rules and regulations of this program have been explained to my satisfaction and I agree to abide by all of them.

Most health insurance companies cover a portion, if not all, of the services provided at **S&B Behavioral Health Care**. I acknowledge that I will be responsible for any fees for the services provided that are not covered by my insurance company. In the event that I do not have health insurance, the fees for **S&B Behavioral Health Care's** services are as follows:

- Initial Assessment = \$192.57
- Level 2.5 (PHP) = \$293.31 per day
- Level 2.1 (IOP) = \$174.60 per day
- Level 1 (OP) = \$54.49 per group session; \$27.93 per individual session per fifteen (15) minutes
- Individual Peer Recovery Support Services \$16.87 per 15 minutes

Should I have difficulties with paying the fees, I acknowledge that **S&B Behavioral Health Care** offers a discount program for assistance with the costs associated with the services needed. I understand that I can fill out an application for the discount program but am not guaranteed to qualify. Should I wish to apply for the discount program, I will speak with the S&B Behavioral Health Care team members.



CONSENT FOR SERVICE FEE AND AGREEMENT

Based on the information provided above, I understand that any fees I am responsible for are expected to be paid weekly, unless other arrangements have been made between the **S&B Behavioral Health Care** staff and myself. Failure to pay timely may result in my dismissal from the program.

Client Signature

Date



CONSENT FOR SPECIMEN COLLECTION AND ANALYSIS

Client Name: _____

MRN: _____

DOB: _____

In accordance with **S&B Behavioral Health Care LLC** guidelines, each client is required to provide random supervised urine specimens to be submitted to an approved testing laboratory for testing. *Failure to provide a random, supervised urine specimen results in the urine specimen being recorded in the client's record as positive for illicit drugs.* Failure to provide requested random, supervised urine specimens might result in a two-four week extension of the program, increased individual counseling sessions, or termination from the program for noncompliance with program guidelines. Failure to provide a valid urine specimen, or should you provide urine specimens that test positive for illicit drugs, may result in a recommendation for a higher level of care or termination from the program for noncompliance with program guidelines.

It is your responsibility to check with a staff member before self-medicating with any prescription medication, including over-the-counter medication. Refusal to sign this Consent to Urine Specimen Collection and Analysis will be grounds to deny admission. Your failure to agree to abide by program rules, federal regulations and the Behavioral Health Administration requirements is unacceptable during the admission process. Initial drug screening is completed at the time of assessment. This test is not meant to be punitive, but rather to obtain a baseline of any prior drug use prior to admission into the **S&B Behavioral Health Care** program. A minimum of two random, observed urinalysis tests are performed every month for PHP. A minimum of one random, observed urinalysis test is performed every week for IOP/OP. In accordance with federal regulations, each random urine specimen is tested for alcohol, marijuana, opiates, PCP, amphetamines, cocaine, barbiturates, buprenorphine, naltrexone and other drugs as determined by the program, such as antidepressants and antipsychotics.

You have the choice of using your health insurance to pay for your toxicology screenings; however, we will not be held responsible for your co-pays (if any) or if your insurance company should deny payment. Should you opt out of using your insurance you will have to **pay S&B Behavioral Health Care** for each toxicology screen you submit.



CONSENT FOR SPECIMEN COLLECTION AND ANALYSIS

Default to:

- ☒ Use my Medicaid insurance for all billing purpose.
- ☐ Do not use my health insurance carrier. I will pay out of pocket.

The cost is \$20.00 per test.

Client Signature

Date



RELEASE AND PERMISSION TO RECORD
SESSIONS & USE CASE MATERIAL

Client Name: _____

MRN: _____

DOB: _____

By signing this consent, you are allowing **S&B Behavioral Health Care LLC** to record therapy sessions. Video and audio recordings are sometimes used as aids in the therapy process, in the education of behavioral health professions, and in research. **S&B Behavioral Health Care** needs to have your written permission to make and use these recordings and materials for these purposes.

Your material may help in the development of the behavioral health field or in the training of healthcare workers. It is possible that **S&B Behavioral Health Care** could use your material in teaching, supervision, consultation with other providers, publishing or scientific research. When **S&B Behavioral Health Care** uses materials from testing or therapy work, **S&B Behavioral Health Care** does not want anyone who hears, reads, or sees it to be able to identify the clients involved. Therefore, **S&B Behavioral Health Care** would conceal your identity by one of the following methods:

1. Reporting the results as grouped data; that is, publishing only numbers like averages, and not publishing any individual's scores or names.
2. Removing, or if this is not possible, greatly changing all names, dates, places, descriptions, or any other information by which you or anyone else involved could be identified.
3. Using any other methods for maintaining confidentiality appropriate to the medium, such as electronically concealing someone's face or altering his or her voice.
4. Using other methods, including those not yet available, that would be consistent with the professional code of ethics and professional guidelines for the maintenance of confidentiality.

These materials will be presented only to other healthcare professionals and to their interns/students. All of these persons are bound by federal and state laws and professional rules about clients' privacy. **S&B Behavioral Health Care** will keep all these materials in a safe location and destroy them as soon as they are no longer needed.



IF YOU DO AGREE/CONSENT

Please check all of the following boxes after reading each one thoroughly:

- ☐ I, the client (or legal representative), do consent to the possible recording of some sessions for the purposes described above.
- ☐ The purpose and value of recording have been fully explained to me and I freely and willingly consent to this recording.
- ☐ I understand that I may ask for the recording to be turned off or erased at any time during my sessions.
- ☐ I further understand that I may then ask for a recording to be destroyed.
- ☐ I understand that I am fully responsible for my own participation in any and all exercises and activities suggested by provider.
- ☐ I agree not to hold the provider legally responsible for the effect of these exercises on me, either during the session or later.
- ☐ I hereby give up my right to any and all interests that I may have in the materials.
- ☒ I agree to let **S&B Behavioral Health Care** to be the sole owner of all the rights in these materials for all purposes described above.



RELEASE AND PERMISSION TO RECORD
SESSIONS & USE CASE MATERIAL

IF YOU DO NOT AGREE

Please check all of the following boxes after reading each one thoroughly:

- ☐ I, the client (or legal representative), do not consent to the possible recording of some sessions for the purposes described above.
- ☐ I understand that since I do not agree to the uses of these materials or the recording of meetings as indicated, I will not be penalized in any way, and it will not affect the care I am to receive in any way.

Client Signature

Date



Client Name: _____

MRN: _____

DOB: _____

In consideration of being given the opportunity to participate in Adventure and/or Activity Therapy, I, for myself, my personal representatives, assigns, heirs, and next of kin:

1. Acknowledge, agree and represent that I understand the nature of the Activities, on land, and that I am qualified, in good health, and in proper physical condition to participate in such Therapy.
2. Fully understand that:
 - a. Adventure/Activity Therapy may involve risks and dangers of serious bodily injury, including permanent disability, paralysis and death ("Risks").
 - b. These Risks and dangers may be caused by my own actions, or inactions, the actions or inactions of others participating in the Therapy, the condition in which the Therapy takes place, or the negligence of the parties named below;
 - c. There may be other risks and social and economic losses either not known to me or not readily foreseeable at this time; and I fully accept and assume all such risks and all responsibility for losses, costs and damages I incur as a result of my participation in the Therapy.
3. Agree and warrant that I will examine and inspect each Therapy in which I take part as a client of **S&B Behavioral Health Care**, that if I observe any condition which I consider to be unacceptably hazardous or dangerous. I will notify the proper authority in charge of the Therapy and will refuse to take part in the Therapy until the condition has been corrected to my satisfaction.
4. Hereby release, discharge, and covenant not to sue all companies, **S&B Behavioral Health Care**, their subsidiaries or related companies, administrators, directors, agents, officers, volunteers and employees, any sponsors, and if applicable, owners and lessors of premises, on which the Therapy takes place (each considered a part of this release) from all liability, claims, demands, losses or damages on my account caused or alleged to be caused in whole or in part by the negligence of the releases.



5. Further agree that if, despite this release and waiver of liability, assumption of risk, and indemnity agreement, I, or anyone on my behalf, makes a claim against any of the parties. I will indemnify, save and hold harmless each of the parties from any litigation's expenses, attorney fees, loss, liability, damage, or cost which may incur as a result of such claim

I have read this agreement, fully understand its terms, understand that have given up substantial rights by signing it and have signed it freely and without any inducement or assurance of any nature and intend it to be a complete and unconditional release of all liability to the greatest extent allowed by law and agree that if any portion of this agreement is held to be invalid, the balance, notwithstanding, shall continue in full force and effect.

Client Signature

Date



CONSENT FOR REPORTING COMMUNICABLE DISEASES

Client Name: _____

MRN: _____

DOB: _____

Maryland statutes provide that any individual entering substance abuse treatment who has been diagnosed with a communicable disease reports this upon admission. I hereby give my permission to release to the Public Health Department, Disease Control Division any information regarding but not limited to the communicable diseases listed below:

HIV/AIDS	Amebiasis	Brucellosis	Animal Bite
COVID-19	Anthrax	Chancroid	Dengue
Botulism	Encephalitis	Gonorrhea	Granuloma
Campylobacteriosis	Hansen's Disease-	Legionnaires Disease	Hemorrhagic Fever
Diphtheria	Leprosy	Malaria	Leptospirosis
Inguinale	Histoplasmosis	Pesticide Poisoning	Measles/Mumps
Hepatitis	Meningococcal	Rabies	Paralytic Shellfish
Lymphogranuloma	Disease	Shigellosis	Plaque
Meningitis	Pertussis	Trichomonas	Relapsing Fever
Poisoning	Psittacosis	Typhoid Fever	Smallpox
Poliomyelitis	Salmonellosis	Vibrio Infections	Typhus
Rubella	Toxoplasmosis		Yellow Fever
Syphilis Tetanus	Tularemia		
Tuberculosis	Cholera		
Vibrio			

Client Signature

Date



Client Name: _____

MRN: _____

DOB: _____

Clients receiving treatment at **S&B Behavioral Health Care LLC** shall have the following rights:

1. The right to be treated with respect and dignity.
2. The right to a safe, sanitary, and humane treatment environment.
3. The right to an environment promoting their overall wellbeing, protecting them from harm, abuse, and neglect.
4. The right to receive treatment regardless of race, color, creed/religion, sexual orientation, gender, national origin, physical or mental disability.
5. The right to receive appropriate compassionate treatment which limits the client's personal freedom only to the extent necessary to the client's treatment or rehabilitation needs and applicable legal requirements.
6. The right to be protected from harm and to be free of mental, physical and sexual abuse at program facility. All allegations of client abuse at the facility by staff members must be reported to the local law enforcement agency or to the Program Director who must report them to the local law enforcement agency.
7. The right to receive information about the ways they can submit complaints or grievances related to client abuse to local law enforcement agencies or to the Program Director, who shall report the incident to the law enforcement agency.
8. The right to participate, in a manner appropriate to the client condition, in the development and periodic reviews of their individualized initial and subsequent treatment and rehabilitation plans.
9. The right to an individualized treatment and/or rehabilitation plan and the right to a continuing care plan.
10. The right to be informed in appropriate terms and language about
 - a. The content, goals and objectives of their treatment.
 - b. The nature and significant possible negative or adverse effects of their treatment or rehabilitation.



- c. The right to receive information from the program staff responsible for carrying out the client's treatment about the program staff's name, title, role, knowledge and experience.
 - d. The right, when appropriate, to receive information about other treatment options, services or providers of behavioral health services.
11. The right to privacy and to expect that all information about care and services received will be confidential, unless restricted by law.
12. The right of the client to have access to their treatment and/or rehabilitation records and the right, with written permission, for the client's attorney to have access to their records. The right of the client to a written summary of sections of the records that is deemed potentially harmful to the client by the client's physician, in cases where the client's physician believes that it would be harmful to the individual to read his records.
13. The right, prior to admission, to receive an explanation in language and terms that the participant can understand about the following:
- a. Admission and discharge policies
 - b. A list of the client's rights posted in a prominent place in the facility
 - c. The charges and fees that the client will be required to pay
14. The right to refuse medication.
15. The right to refuse a continuing care plan.
16. The right to refuse to participate in physically intrusive research.
17. The right to file a grievance if the client is not satisfied with the treatment or rehabilitation that they receive.

ACKNOWLEDGEMENT

Client Signature

Date



Client Name: _____

MRN: _____

DOB: _____

S&B Behavioral Health Care LLC will treat what you tell us with great care. Our professional ethics (that is, our profession's rules about moral matters) and the laws of this state prevent us from telling anyone else what you tell us unless you give us written permission. These rules and laws are the ways our society recognizes and supports the privacy of what we talk about in other words- the "confidentiality". However, **S&B Behavioral Health Care LLC** cannot promise that everything you tell us will never be revealed to someone else. There are some times when the law requires us to tell things to others. There are also some other limits on our confidentiality. We need to discuss these, because we want you to understand clearly what we can and cannot keep secret.

These are very important issues, so please read these pages carefully and keep this copy.

1. When you or other persons are in physical danger, the law requires us to tell others about it. Specifically:
 - a. If **S&B Behavioral Health Care** comes to believe that you are threatening serious harm to another person, **S&B Behavioral Health Care** is required to try to protect that person. We may have to tell the person and the police or perhaps try to have you taken to the hospital.
 - b. If you seriously threaten or act in a way that is highly likely to harm yourself. **S&B Behavioral Health Care** may have to seek a hospital for you, or to call on your family members or others who can help protect you. If such a situation does come up. **S&B Behavioral Health Care** will fully discuss the situation with you before we do anything unless there is a very strong reason not to.
 - c. In an emergency where your life or health is in danger, and **S&B Behavioral Health Care** cannot get your consent, **S&B Behavioral Health Care** may give another professional some information to protect your life. We will try to get your permission first, and **S&B Behavioral Health Care** will discuss this with you as soon as possible afterwards.
 - d. If **S&B Behavioral Health Care** believes or suspects that you are abusing a child, an elderly person, or a disabled person, **S&B Behavioral Health Care** must file a



report with a state agency. To "abuse" means neglect, hurt, or sexually molest another person. **S&B Behavioral Health Care** does not have any legal power to investigate the situation to find out all of the facts. The state agency will investigate. If this might be your situation, we should discuss the legal aspects in detail before you tell us anything about these topics. You may also want to talk to your lawyer.

- e. If you disclose to **S&B Behavioral Health Care** that you yourself are or were being abused as a child or as a vulnerable adult, **S&B Behavioral Health Care** may need to report this information to the appropriate state agency. We should discuss mandatory reporting of disclosed abuse before you disclose any abuse that you may have experienced to us.
 - f. In any of these situations, **S&B Behavioral Health Care** would reveal only the information that is needed to protect you or the other person. We would not tell everything you have told us.
2. In general, if you become involved in a court case or proceeding, you can prevent us from testifying in court about what you have told us. This is called "privilege" and it is your choice to prevent us from testifying or to allow us to do so. However, there are some situations where a judge or court may require us to testify:
- a. In child custody or adoption proceedings, where your fitness as a parent is questioned or in doubt
 - b. In cases where your emotional or mental condition is important for a court's decision
 - c. During a malpractice case or an investigation of us or another provider by a professional group
 - d. In a civil commitment hearing to decide if you will be admitted to a psychiatric hospital
 - e. When you are seeing us for court ordered evaluations or treatment
 - i. In this case we need to discuss confidentiality fully, because you don't have to tell us what you don't want the court to find out through our report.
3. There are a few other things you must know about confidentiality and your treatment:
- a. **S&B Behavioral Health Care** may sometimes consult (talk) with another professional about your treatment. This other person is also required by



professional ethics to keep your information confidential. Likewise, when a staff member is unavailable, another provider will be available to help our clients. **S&B Behavioral Health Care** must give him or her some information about our clients, like you.

- b. **S&B Behavioral Health Care** is required to keep records of your treatment, such as the notes **S&B Behavioral Health Care** takes when we meet. You have a right to review these records with us. If something in the record might seriously upset you, **S&B Behavioral Health Care** may leave it out, but **S&B Behavioral Health Care** will fully explain our reasons to you.
4. Here is what you need to know about confidentiality in regard to insurance and money matters:
 - a. If you use your health insurance to pay a part of our fees, insurance companies require some information about our services. Insurers, such as Blue Cross/Blue Shield or other companies, usually want only your diagnosis, our fees, the dates we met, and sometimes a treatment plan. Managed care organizations, however, ask for much more information about you and your symptoms, as well as a detailed treatment plan.
 - b. It is against the law for insurers to release information about our office visits to anyone without your written permission. Although **S&B Behavioral Health Care** believes the insurance company will act morally and legally, **S&B Behavioral Health Care** cannot control who sees this information at the insurer's office. You cannot be required to release more information just to get payments.
 - c. If you have been referred here by another agency they may require information from us. In order to give them any information you will sign a release. Again, **S&B Behavioral Health Care** believes that agencies will act morally and discuss our agreement with your employer or the program before we talk further.
5. Confidentiality in group therapy is also a special situation.
 - a. In group therapy, the other members of the group are not counselors. They do not have the same ethics and laws that **S&B Behavioral Health Care** have to work under. You cannot be certain that they will always keep what you say in the group confidential.



6. Finally, here are a few other points.

- a. **S&B Behavioral Health Care** will not record our therapy sessions on audiotape or videotapes without your written permission.
- b. If you want us to send information about our services with you to someone else. You must sign a "release of records" form. **S&B Behavioral Health Care** have copies which you can see so you will know what is involved.

The laws and rules on confidentiality are complicated. Situations that are not mentioned here come up only rarely in our practice. Please bear in mind that **S&B Behavioral Health Care** is not able to give legal advice. If you have special or unusual concerns, and so need special advice, **S&B Behavioral Health Care** strongly suggests that you talk to a lawyer to protect your interests legally. The signatures here show that we each have read, discussed, understand, and agree to abide by the points presented above.

ACKNOWLEDGEMENT

Client Signature

Date



DISCLOSURE OF CLIENT RECORDS/PRESENCE RESTRICTIONS

Client Name: _____

MRN: _____

DOB: _____

The client records to which these regulations apply may be disclosed or used only as permitted by these regulations and may not otherwise be disclosed or used in any civil, criminal, administrative, or legislative proceedings conducted by any federal, state, or local authority. Any disclosure made under these regulations must be limited to that information which is necessary to carry out the purpose of the disclosure. It is understood that the general limits to confidentiality apply:

- Harm to self or others
- Judicial Order (including warrants)
- Mandatory reporting of abuse/endangerment of children, elderly, or disabled
- In the event of a signed Release of Information.

UNCONDITIONAL COMPLIANCE REQUIRED

The restrictions on disclosure and use in the regulations apply whether the holder of the information believes that the person seeking the information already has it, has other means of obtaining it, is law enforcement or other official, has obtained a subpoena, or asserts any other justification for a disclosure or use which is not permitted by these regulations.

ACKNOWLEDGING THE PRESENCE OF CARE: RESPONDING TO REQUEST

The presence of an identified client in the facility or component of the facility which is publicly identified as a place where only alcohol or drug abuse diagnosis, treatment, or referral is provided, may be acknowledged only if the client's written consent is obtained in accordance with these regulations. The regulations permit acknowledgment of the presence of an identified client in a facility, or part of a facility, if the facility is not publicly identified as only an alcohol or drug abuse diagnosis, treatment or referral facility, and if the acknowledgment does not reveal that the client is an alcohol or drug abuse user.

Any answer to a request for a disclosure of client records, which is not permissible under these regulations, must be made in a way that will not affirmatively reveal that an unidentified individual has been, or is being diagnosed or treated for alcohol or drug abuse. An inquiring party may be



DISCLOSURE OF CLIENT RECORDS/PRESENCE RESTRICTIONS

given a copy of these regulations and advised that they restrict the disclosure of alcohol or drug abuse client records, but may not be told affirmatively that the regulations restrict.

the disclosure of the records of an identified client. The regulations do not restrict a disclosure that an identified individual is not and never has been a client.

I hereby acknowledge the receipt of and understand the Confidentiality Restriction statement as indicated by my signature.

ACKNOWLEDGEMENT

Client Signature

Date



Client Name: _____
MRN: _____
DOB: _____

THIS NOTICE DESCRIBES HOW CLINICAL/MEDICAL INFORMATION ABOUT YOU MAY BE USED AND DISCLOSED AND HOW YOU CAN GET ACCESS TO THIS INFORMATION. PLEASE REVIEW IT CAREFULLY.

This Notice of Privacy Practices describes how we may use and disclose your protected health information (PHI) to carry out treatment, payment or healthcare operations and for other purposes that are permitted or required by law. It also describes your rights to access and control your protected health information. "Protected health information" is information about you, including demographic information, that may identify you and that relates to your past, present or future physical/mental health or condition and related healthcare services.

Uses and Disclosures of Protected Health Information

Your protected health information may be used and disclosed by your provider, our office staff and others outside of our office that are involved in your care and treatment for the purpose of providing healthcare services to you, to pay your healthcare bills, to support the operation of the organization, and any other use required by law.

TREATMENT

We will use and disclose your protected health information to provide, coordinate, or manage your healthcare and any related services. This includes the coordination or management of your healthcare with a third party. For example, we would disclose your protected health information, as necessary, to a home health agency that provides care to you. For example, your protected health information may be provided to a physician to whom you have been referred to ensure that the physician has the necessary information to diagnose or treat you.

PAYMENT

Your protected health information will be used, as needed, to obtain payment for your healthcare services. For example, obtaining approval for a hospital stay may require that your relevant protected health information be disclosed to the health plan to obtain approval for the hospital admission.



HEALTHCARE OPERATIONS

We may use or disclose, as needed, your protected health information in order to support the business activities of our organization. These activities include, but are not limited to, quality assessment activities, employee review activities, training of interns/students, licensing, and conducting or arranging for other business activities. For example, we may disclose your protected health information to interns/students that see clients at our office. In addition, we may use a sign-in sheet at the registration desk where you will be asked to sign your name and indicate your provider. We may also call you by name in the waiting room when your provider is ready to see you. We may use or disclose your protected health information, as necessary, to contact you to remind you of your appointment.

We may use or disclose your protected health information in the following situations without your authorization. These situations include: as required by law, public health issues as required by law, communicable diseases, health oversight, abuse or neglect, Food and Drug Administration requirements, legal proceedings, law enforcement, coroners, funeral directors, organ donation, research, criminal activity, military activity and national security, workers' compensation, inmates, required uses and disclosures. Under the law, we must make disclosures to you upon your request. Under the law, we must also disclose your protected health information when required by the Secretary of the Department of Health and Human Services to investigate or determine our compliance with the requirements. Other permitted and required uses and disclosures will be made only with your consent, authorization or opportunity to object unless required by law. You may revoke this authorization, at any time, in writing, except to the extent that your provider or the organization has taken an action in reliance on the use or disclosure indicated in the authorization.



Your Rights

The following is a statement of your rights with respect to your protected health information.

- You have the right to inspect and copy your protected health information. Under federal law, however, you may not inspect or copy the following records: psychotherapy notes, information compiled in reasonable anticipation of, or use in, a civil, criminal, or administrative action or proceeding, and protected health information that is subject to law that prohibits access to protected health information.
- You have the right to request a restriction of your protected health information. This means you may ask us not to use or disclose any part of your protected health information for the purposes of treatment, payment or healthcare operations. You may also request that any part of your protected health information not be disclosed to family members or friends who may be involved in your care or for notification purposes as described in this Notice of Privacy Practices. Your request must state the specific restriction requested and to whom you want the restriction to apply. Your provider is not required to agree to a restriction that you may request. If the provider believes it is in your best interest to permit use and disclosure of your protected health information, your protected health information will not be restricted. You then have the right to use another healthcare professional.
- You have the right to request to receive confidential communications from us by alternative means or at an alternative location.
- You have the right to obtain a paper copy of this notice from us, upon request, even if you have agreed to accept this notice alternatively (i.e. electronically).
- You may have the right to have your provider amend your protected health information. If we deny your request for amendment, you have the right to file a statement of disagreement with us and we may prepare a rebuttal to your statement and will provide you with a copy of any such rebuttal.
- You have the right to receive an accounting of certain disclosures we have made, if any, of your protected health information. We reserve the right to change the terms of this notice and will inform you by mail of any changes. You then have the right to object or withdraw as provided in this notice.



Complaints

You may complain to us or to the Department of Health and Human Services if you believe your privacy rights have been violated by us. You may file a complaint with us by notifying our Compliance Officer of your complaint. We will not retaliate against you for filing a complaint.

This notice was published and became effective on 06/01/2022. We are required by law to maintain the privacy of, and provide individuals with, this notice of our legal duties and privacy practices with respect to protected health information. If you have any objections to this form, please ask to speak with our Compliance Officer in person or by phone at our Main Phone Number.

I acknowledge that I have read and received a copy of this Notice of Privacy Practices.

Client Signature

Date



Client Name: _____

MRN: _____

DOB: _____

General Criteria

After arrival at **S&B Behavioral Health Care LLC**, clients must remain on the premises. If a client leaves without permission, the client will be immediately discharged from the program.

The following behaviors will result in immediate discharge:

- Bringing a gun, firearms, knives or any other weapons onto the program premises
- Bringing alcohol, or other drugs, or any other illegal substances onto the premises
- Fighting with other clients or staff members
- Destruction of other client's property or the program's property or materials
- Breaking confidentiality of any client involved with the program

Any client who has signed a behavioral contract must adhere to the conditions of the contract or be discharged.

Partial Hospitalization, Intensive Outpatient Program and Outpatient Program:

1. Each client shall undergo random urine drug screens and/or breath tests (for alcohol) during the program. If a client tests positive for any mood altering chemical on one occasion, while a client of the Partial Hospitalization, Intensive Outpatient Program or Outpatient Program, further treatment recommendations will be made. If a client refuses to be tested, then it will be considered a positive test.
2. **S&B Behavioral Health Care** will assign homework and other activities (such as meetings) to be performed outside of the attendance at **S&B Behavioral Health Care** and it is understood that the client is obligated to complete such assignments by the assigned due date. The client understands that **S&B Behavioral Health Care** has determined that this is an essential part of the program to help the client better address their addiction and other issues and that it is required by the program.



CRITERIA FOR DISCHARGE

By signing below, the client agrees that if they have difficulty completing the assignments, they will discuss this with their provider. They also understand that if the issue becomes chronic and they are not completing their assignments, that **S&B Behavioral Health Care** may and will discharge them from the program.

Client Signature

Date



Client Name: _____

MRN: _____

DOB: _____

All clients of **S&B Behavioral Health Care LLC** are entitled to be treated according to its foundational principles and applicable state and federal laws and regulations. A client who believes they have been treated unfairly, does not agree with their termination from the program or change of status within the program, and/or has a complaint of any kind is entitled to have their grievance heard by the staff/supervisory personnel and, if necessary, the Executive team. The program or a staff member may also bring a grievance against a client for reasons including, but not limited to, the client failing to abide by the program's rules.

This grievance may result in the client's discharge from the program or change of status within the program. Clients shall be immediately notified in writing of any decision that results in their discharge from or change of status within the program and shall have the right to appeal this decision by filing a grievance of their own within five (5) working days from the date they received written notice. Under no circumstance shall any employee of **S&B Behavioral Health Care** retaliate against any client who files a grievance. If significant changes are made to this policy, which result in there being a substantial difference to what is stated herein, you will be notified of such as long as you are an active client of the program. Partial Hospitalization, Intensive Outpatient Program and Outpatient Program:

Summary of Grievance Process:

- If you feel at any time you are being treated unfairly as a member of this program, if you do not agree with termination from or status change within the program, and/or if you have complaints of any kind, please bring this matter to the attention of your counselor or any available staff member.
- If the matter cannot be worked through informally, you may request a Grievance Form from your counselor or any staff member and obtain their assistance in completing the form.
- If your grievance is with a particular staff member, the form will be sent to the staff member's immediate supervisor who will meet with the staff member within one week and



take appropriate action if necessary. The results of the meeting and any action taken will be reported.

- If your grievance is with the program in general, or your termination from or change of status within the program, the form will be sent to the appropriate department manager responsible for that portion of the program. That department manager will investigate the matter and take appropriate action, if necessary, within one week. You will be notified of any action taken.
- If you are still not satisfied, the complaint will be forwarded to the program director or other appropriate person for further action. You will be informed of any action taken.
- If you are still not satisfied, you may write to the executive team of **S&B Behavioral Health Care** for final resolution. You will be notified of any action taken by the executive team.

Grievance Protocols

Clients are encouraged to speak to **S&B Behavioral Health Care** staff (administrative, professional or support personnel) if they are not satisfied with an aspect of their treatment by staff or the environment in which services and/or housing is provided. **S&B Behavioral Health Care** staff will respond to the client's concern(s) with two (2) business days. If the concern is not resolved to the client's satisfaction after talking with the staff, the client is encouraged to use the grievance procedures outlined below. A written notice of the grievance procedure is provided to each client

1. All clients have the right and are encouraged to communicate their grievance to a **S&B Behavioral Health Care** staff member or organization representative. There will be no consequences or retaliation for the client filing a grievance
2. All clients have a right to file a formal written grievance. The client may request a form from any staff member, as well as obtain a copy from the designated house binder if they live in supportive housing. The client should fill out the forms and return it to any staff member.
3. If the client is uncomfortable with filling out the grievance form on their own, the client may request any staff member to assist them.
4. Written grievances shall be forwarded to the **S&B Behavioral Health Care** Administrator/Compliance department. If the client needs assistance with the form or in



composing the complaint/grievance, assistance will be provided by the **S&B Behavioral Health Care** team or the client's representative of choice.

5. In the instance where the decision maker is the subject of a grievance, decision making authority shall be delegated to the CEO.
6. At any point in this process, the client may request an advocate for assistance with this process.
7. Time frame for expedient resolution of grievances is two (2) business days upon receipt of the complaint/grievance by the **S&B Behavioral Health Care** Administrator/Compliance department.
8. The client will be sent a written notice of the grievance outcome and steps for appealing the outcome from the Administrator/Compliance department.
9. Throughout this process, the client has the right to contact, make a complaint and/or appeal the outcome to the:



Maryland Department of Health*Behavioral Health Administration*

55 Wade Ave, Catonsville, MD 21228

As part of the **S&B Behavioral Health Care** Quality Assurance program, a confidential summary of client complaints/grievances prepared by the Administrator/Compliance department are reviewed by the executive team on a quarterly basis, so as to improve efficiency and client satisfaction. All grievances filed will be reported in a confidential summary prepared by the

Administrator/Compliance department. All client complaint/grievance forms, investigation documentation and written resolution determinations will be maintained by the Administrator/Compliance department. All summary documentation for the Quality Assurance Program will be maintained by the Administrator/Compliance department, including any returned notices. The Administrator/Compliance department annually reviews the grievance policies and procedures and makes any necessary adjustments to improve both services and processes overall.

Client Signature

Date

CLIENT ORIENTATION HANDBOOK ACKNOWLEDGMENT

Client Name: _____

MRN: _____

DOB: _____

This is to confirm I have received, reviewed and understand the information contained in the **S&B Behavioral Health Care LLC** Client Orientation Handbook

- Information reviewed included the following:
- Program description
- Program rules
- Privacy practices
- Universal precautions for HIV
- Client's rights
- Grievance protocols
- Urinalysis protocols

I acknowledge that I have been assigned to group therapy sessions:

I understand that I must abide by the group therapy attendance protocol, detailed within the client handbook. If I am unable to attend an assigned group therapy session. I must notify my primary counselor at least 24 hours prior to the group therapy session. If I need to change my group therapy session assignment, I will speak with my primary counselor first to obtain approval.

Client Signature

Date



Client Name: _____

MRN: _____

DOB: _____

ABOUT HIV/AIDS

HIV is a virus spread through blood and certain other bodily fluids that attacks the body's immune system, specifically the CD4 cells, often called T cells. Over time, HIV can destroy so many of these cells that the body can't fight off infections and disease. These special cells help the immune system fight off infections. Untreated, HIV reduces the number of CD4 cells (T cells) in the body. This damage to the immune system makes it harder and harder for the body to fight off infections and some other diseases. Opportunistic infections or cancers take advantage of a very weak immune system and signal that the person has AIDS. Learn more about the stages of HIV and how to know whether you're infected.

HIV TRANSMISSION

Myths persist about how HIV is transmitted. This section provides the facts about HIV risk from different types of sex, injection drug use, and other activities.

How is HIV passed from one person to another?

You can get or transmit HIV only through specific activities. Most commonly, people get or transmit HIV through sexual behaviors and needle or syringe use. Only certain bodily fluids such as blood, semen, pre-seminal fluid, rectal fluids, vaginal fluids, and breast milk from a person who has HIV can transmit HIV. These fluids must come in contact with a mucous membrane or damaged tissue or be directly injected into the bloodstream (from a needle or syringe) for transmission to occur. Mucous membranes are found inside the rectum, vagina, penis, and mouth.

In the United States, HIV is spread mainly by:

- Having anal or vaginal sex with someone who has HIV without using a condom or taking medicines to prevent or treat HIV.
 - Anal sex is the highest risk sexual behavior. Being the receptive partner (bottom) is riskier than being the insertive partner (top).
 - Vaginal sex is the second highest risk sexual behavior.
- Sharing needles or syringes, rinse water, or other equipment (works) used to prepare drugs for injection with someone who has HIV.



- HIV can live in a used needle for up to 42 days depending on temperature and other factors.

Less commonly, HIV may be spread:

- From mother to child during pregnancy, birth, or breastfeeding
 - Although the risk can be high if a mother is living with HIV and not taking medicine, recommendations to test all pregnant women for HIV and start HIV treatment immediately have lowered the number of babies who are born with HIV.
- By being stuck with an HIV contaminated needle or other sharp object
 - This is a risk mainly for health care workers.
- Oral sex, putting the mouth on the penis (fellatio), vagina (cunnilingus), or anus (rimming)
 - In general, there's little to no risk of getting HIV from oral sex. However, transmission of HIV, though extremely rare, is theoretically possible if an HIV positive man ejaculates in his partner's mouth during oral sex.
- Receiving blood transfusions, blood products, or organ/tissue transplants that are contaminated with HIV.
 - This was more common in the early years of HIV, but now the risk is extremely small because of rigorous testing of the US blood supply and donated organs and tissues.
- Eating food that has been pre-chewed by an HIV infected person.
 - The contamination occurs when infected blood from a caregiver's mouth mixes with food while chewing. The only known cases are among infants.
- Being bitten by a person with HIV
 - Each of the very small number of documented cases has involved severe trauma with extensive tissue damage and the presence of blood. There is no risk of transmission if the skin is not broken.
- Contact between broken skin, wounds, or mucous membranes and HIV infected blood or blood contaminated body fluids.
- Deep, open mouth kissing if both partners have sores or bleeding gums and blood from the HIV positive partner gets into the bloodstream of the HIV negative partner.
 - HIV is not spread through saliva.



How well does HIV survive outside the body?

HIV does not survive long outside the human body (such as on surfaces), and it cannot reproduce outside a human host.

It is not spread by:

- Mosquitoes, ticks, or other insects
- Saliva, tears, or sweat that is not mixed with the blood of an HIV positive person.
- Hugging, shaking hands, sharing toilets, sharing dishes, or closed mouth or "social" kissing with someone who is HIV positive.
- Other sexual activities that don't involve the exchange of body fluids (for example, touching)

Can I get HIV from anal sex?

In fact, having anal sex is the riskiest type of sex for getting or spreading HIV. HIV can be found in the blood, semen, pre-seminal fluid, or rectal fluid of a person infected with the virus. The bottom is at greater risk of getting HIV because the lining of the rectum is thin and may allow HIV to enter the body during anal sex, but the top is also at risk because HIV can enter through the opening of the penis or through small cuts, abrasions, or open sores on the penis.

Can I get HIV from vaginal sex?

Vaginal sex is the sexual behavior with the second highest risk for getting or transmitting HIV. It is possible for either partner to get HIV from vaginal sex. When a woman has vaginal sex with a partner who's HIV positive, HIV can enter her body through the mucous membranes that line the vagina and cervix. Most women who get HIV get it from vaginal sex. Men can also get HIV from having vaginal sex with a woman who's HIV positive. This is because vaginal fluid and blood can carry HIV. Men get HIV through the opening at the tip of the penis (or urethra), the foreskin if they're not circumcised, or small cuts, scratches, or open sores anywhere on the penis.

Can I get HIV from oral sex?

The chance that an HIV negative person will get HIV from oral sex with an HIV positive partner is extremely low. Oral sex involves putting the mouth on the penis (fellatio), vagina (cunnilingus), or anus (anilingus). In general, there's little to no risk of getting or transmitting HIV through oral sex. Factors that may increase the risk of transmitting HIV through oral sex are ejaculation in the mouth with oral ulcers, bleeding gums, genital sores, and the presence of other sexually transmitted



diseases (STDs), which may or may not be visible. You can get other STDs from oral sex. Also, if you get feces in your mouth during anilingus, you can get hepatitis A and B, parasites like Giardia, and bacteria like Shigella, Salmonella, Campylobacter, and E.coli.

Is there a connection between HIV and other sexually transmitted infections?

Having another sexually transmitted disease (STD) can increase the risk of getting or transmitting HIV. If you have another STD, you're more likely to get or transmit HIV to others. Some of the most common STDs include gonorrhea, chlamydia, syphilis, trichomoniasis, human papillomavirus (HPV), genital herpes, and hepatitis. The only way to know for sure if you have an STD is to get tested. If you're sexually active, you and your partners should get tested for STDs (including HIV if you're HIV negative) regularly, even if you don't have symptoms. If you are HIV negative but have an STD, you are about 3 times as likely to get HIV if you have unprotected sex with someone who has HIV.

There are two ways that having an STD can increase the likelihood of getting HIV. If the STD causes irritation of the skin (for example, from syphilis, herpes, or human papillomavirus), breaks or sores may make it easier for HIV to enter the body during sexual contact. Even STDs that cause no breaks or open sores (for example, chlamydia, gonorrhea, trichomoniasis) can increase your risk by causing inflammation that increases the number of cells that can serve as targets for HIV. If you are HIV positive and also infected with another STD, you are about 3 times as likely as other HIV infected people to spread HIV through sexual contact. This appears to happen because there is an increased concentration of HIV in the semen and genital fluids of HIV positive people who also are infected with another STD.

Does my HIV positive partner's viral load affect my risk of getting HIV?

Yes, as an HIV positive person's viral load goes down, the chance of transmitting HIV goes down. Viral load is the amount of HIV in the blood of someone who is HIV positive. When the viral load is very low, it is called viral suppression. Undetectable viral load is when the amount of HIV in the blood is so low that it can't be measured. In general, the higher someone's viral load, the more likely that person is to transmit HIV. People who have HIV but are in care, taking HIV medicines, and have a very low or undetectable viral load are much less likely to transmit HIV than people who have HIV and do not have a low viral load.



However, a person with HIV can still potentially transmit HIV to a partner even if they have an undetectable viral load, because:

- HIV may still be found in genital fluids (i.e., semen, vaginal fluids). The viral load test only measures the virus in the blood.
- A person's viral load may go up between tests. When this happens, they may be more likely to transmit HIV to partners.
- Sexually transmitted diseases increase viral load in genital fluids.

If you're HIV positive, getting into care and taking HIV medicines (called antiretroviral therapy or ART) the right way, every day will give you the greatest chance to get and stay virally suppressed, live a longer, healthier life, and reduce the chance of transmitting HIV to your partners. If you're HIV negative and have an HIV positive partner, encourage your partner to get into care and engage in HIV treatment. Taking other actions, like using a condom the right way every time you have sex or taking daily medicine to prevent HIV if -you're HIV negative, can lower your chances of transmitting or getting HIV even more.

Can I get HIV from injecting drugs?

Your risk for getting HIV is very high if you use needles or works (such as cookers, cotton, or water) after someone with HIV has used them. People who inject drugs, hormones, steroids, or silicone can get HIV by sharing needles or syringes and other injection equipment. The needles and equipment may have someone else's blood in them, and blood can transmit HIV. Likewise, you're at risk for getting hepatitis B and C if you share needles and works because these infections are also transmitted through blood.

Another reason people who inject drugs can get HIV (and other sexually transmitted diseases) is that when people are high, they're more likely to have risky sex. Stopping injection and other drug use can lower your chances of getting HIV a lot. You may need help to stop or cut down using drugs, but many resources are available. If you keep injecting drugs, you can lower your risk for getting HIV by using only new, sterile needles and works each time you inject. Never share needles or works.



Can I get HIV from using other kinds of drugs?

When you're drunk or high, you're more likely to make decisions that put you at risk for HIV, such as having sex without a condom. Drinking alcohol, particularly binge drinking, and using "club drugs" like ecstasy, ketamine, GHB, and poppers can alter your judgment, lower your inhibitions, and impair your decisions about sex or other drug use. You may be more likely to have unplanned and unprotected sex, have a harder time using a condom the right way every time you have sex, have more sexual partners, or use other drugs, including injection drugs or meth. Those behaviors can increase your risk of exposure to HIV.

If you have HIV, they can also increase your risk of spreading HIV to others. Being drunk or high affects your ability to make safe choices. If you're going to a party or another place where you know you'll be drinking or using drugs, you can bring a condom so that you can reduce your risk if you have vaginal or anal sex. Therapy, medicines, and other methods are available to help you stop or cut down on drinking or using drugs. Talk with a counselor, doctor, or other health care provider about options that might be right for you.

I already have HIV; can I get another kind of HIV?

This is called HIV superinfection. HIV superinfection is when a person with HIV gets infected with another strain of the virus. The new strain of HIV can replace the original strain or remain along with the original strain. The effects of superinfection differ from person to person. Superinfection may cause some people to get sicker faster, because they become infected with a new strain of the virus that is resistant to the medicine (antiretroviral therapy or ART) they're taking to treat their original infection. Research suggests that a hard to treat superinfection is rare. Taking medicine to treat HIV (ART) may reduce someone's chance of getting a superinfection.

Are healthcare workers at risk of getting HIV on the job?

The risk of health care workers being exposed to HIV on the job (occupational exposure) is very low, especially if they use protective practices and personal protective equipment to prevent HIV and other bloodborne infections. For healthcare workers on the job, the main risk of HIV transmission is from being stuck with an HIV contaminated needle or other sharp object. However, even this risk is small. Scientists estimate that the risk of HIV infection from being stuck with a needle used on an HIV infected person is less than 1%.



Can I get HIV from receiving medical care?

Although my transmission is possible in healthcare settings, it is extremely rare. Careful practice of infection control, including universal precautions (using protective practices and personal protective equipment to prevent HIV and other bloodborne infections), protects patients as well as healthcare providers from possible HIV transmission in medical and dental offices and hospitals. The risk of getting HIV from receiving blood transfusions, blood products, or organ/tissue transplants that are contaminated with HIV is extremely small because of rigorous testing of the US blood supply and donated organs and tissues. It is important to know that you cannot get HIV from donating blood. Blood collection procedures are highly regulated and safe.

Can I get HIV from casual contact ("social kissing," shaking hands, hugging, using a toilet, drinking from the same glass, or the sneezing and coughing of an infected person)?

No. HIV is not transmitted:

- By hugging, shaking hands, sharing toilets, sharing dishes, or closed mouth/"social" kissing with someone who is HIV positive.
- Through saliva, tears, or sweat that is not mixed with the blood of an HIV positive person.
- By mosquitoes, ticks or other blood sucking insects
- Through the air

Only certain bodily fluids such as blood, semen, pre-seminal fluid, rectal fluids, vaginal fluids, and breast milk from an HIV infected person can transmit HIV. Most commonly, people get or transmit HIV through sexual behaviors and needle or syringe use. Babies can also get HIV from an HIV positive mother during pregnancy, birth, or breastfeeding.

Can I get HIV from a tattoo or a body piercing?

There are no known cases in the United States of anyone getting HIV this way. However, it is possible to get HIV from a reused or not properly sterilized tattoo, piercing needle, other equipment, or from contaminated ink. It's possible to get HIV from tattooing or body piercing if the equipment used for these procedures has someone else's blood in it or if the ink is shared. The risk of getting HIV this way is very low, but the risk increases when the person doing the procedure is unlicensed, because of the potential for unsanitary practices such as sharing needles or ink. If



you get a tattoo or a body piercing, be sure that the person doing the procedure is properly licensed and that they use only new or sterilized needles, ink, and other supplies.

Can I get HIV from being spit on or scratched by an HIV infected person?

No. HIV isn't spread through saliva, and there is no risk of transmission from scratching because no bodily fluids are transferred between people.

Can I get HIV from mosquitoes?

No. HIV is not transmitted by mosquitoes, ticks, or any other insects.

Can I get HIV from food?

You can't get HIV from consuming food handled by an HIV infected person. Even if the food contained small amounts of HIV infected blood or semen, exposure to the air, heat from cooking, and stomach acid would destroy the virus. Though it is very rare, HIV can be spread by eating food that has been pre-chewed by an HIV infected person. The contamination occurs when infected blood from a caregiver's mouth mixes with food while chewing. The only known cases are among infants.

Are lesbians or other women who have sex with women at risk for HIV?

Case reports of female to female transmission of HIV are rare. The well documented risk of female to male transmission shows that vaginal fluids and menstrual blood may contain the virus and that exposure to these fluids through mucous membranes (in the vagina or mouth) could potentially lead to HIV infection.

Is the risk of HIV different for different people?

Some groups of people in the United States are more likely to get HIV than others because of many factors, including the status of their sex partners, their risk behaviors, and where they live. When you live in a community where many people have HIV infection, the chances of having sex or sharing needles or other injection equipment with someone who has HIV are higher. Gay and bisexual men have the largest number of new diagnoses in the United States. Blacks/African Americans and Hispanics/Latinos are disproportionately affected by HIV compared to other racial and ethnic groups. Also, transgender women who have sex with men are among the groups at highest risk for HIV infection, and injection drug users remain at significant risk for getting HIV.



Risky behaviors, like having anal or vaginal sex without using a condom or taking medicines to prevent or treat HIV, and sharing needles or syringes play a big role in HIV transmission. Anal sex is the highest risk sexual behavior. If you don't have HIV, being a receptive partner (or bottom) for anal sex is the highest risk sexual activity for getting HIV. If you do have HIV, being the insertive partner (or top) for anal sex is the highest risk sexual activity for transmitting HIV. However, there are more tools available today to prevent HIV than ever before. Choosing less risky sexual behaviors, taking medicines to prevent and treat HIV, and using condoms with lubricants are all highly effective ways to reduce the risk of getting or transmitting HIV.

I acknowledge that I have read and understood the information pertaining to HIV/AIDS and have had the chance to ask any and all questions.

Client Signature

Date



Client Name: _____
 MRN: _____
 DOB: _____

S&B Behavioral Health Care LLC has adopted a policy regarding the use of medical marijuana in **S&B Behavioral Health Care** facilities and while engaging in treatment services, except for under certain conditions detailed below. Based on a comprehensive evaluation that took into consideration the medical, ethical, legal and scientific evidence available, **S&B Behavioral Health Care** does not permit the use of medical marijuana on **S&B Behavioral Health Care** premises, which includes supportive living residences, and does not permit **S&B Behavioral Health Care** providers to endorse the initiation of medical marijuana.

What does this mean for clients and providers at S&B Behavioral Health Care facilities?

Under our policy, **S&B Behavioral Health Care** providers may not endorse the initiation of medical marijuana to any client receiving treatment services at **S&B Behavioral Health Care** facilities. Medical marijuana is not allowed on **S&B Behavioral Health Care** property, including the supportive living residences. If a client possesses medical marijuana on **S&B Behavioral Health Care** premises, staff will ask the client to arrange for its immediate removal.

Under our policy, any client who has been provided an MMCC ID card and of whom uses medical marijuana prior to admission into a **S&B Behavioral Health Care LLC** program, may continue their treatment regimen as long as they abide by the following guidelines:

- Clients with an MMCC ID card, who are registered and certified to use medical marijuana, may not reside in the **S&B Behavioral Health Care** supportive living;
- Clients may not use medical marijuana on any property owned or operated by **S&B Behavioral Health Care** and its direct affiliates;
- Clients may not come to a **S&B Behavioral Health Care** facility and/or engage in treatment services with the aroma of medical marijuana;
- Clients may not come to a **S&B Behavioral Health Care** facility and/or engage in treatment services clearly/distinctly under the influence of medical marijuana;



- Clients will be required to provide a legitimate copy of their MMCC ID card to the **S&B Behavioral Health Care** staff to place in their record:
- Clients will be required to provide a copy of their medical marijuana prescription to their primary provider to place in their record.
- Clients will be required to produce any and all receipts from dispensaries to their primary provider to place in their records.

Under our policy, any client who is receiving treatment services **at S&B Behavioral Health Care** facilities, and of whom **has not** been provided an MMCC ID card prior to admission, is **not permitted** to initiate treatment with medical marijuana.

Client Signature

Date



Client Name: _____
 MRN: _____
 DOB: _____

IF YOU SUSPECT AN OVERDOSE

An opioid overdose requires immediate medical attention. An essential first step is to get help from someone with medical expertise as soon as possible.

Call 911 immediately if you or someone you know exhibits any of the symptoms listed below: All you have to say: "Someone is unresponsive and not breathing "Give a clear address and/or description of your location.

Provide Naloxone if certified to administer.

Provide rescue breathing until help arrives, if trained in rescue breathing

SIGNS AND SYMPTOMS OF OVERDOSE OR OVERMEDICATION

Signs of **OVERDOSE**, which is a life-threatening emergency, include:

- Face is extremely pale and/or clammy to the touch
- Body is limp
- Fingernails or lips have a blue or purple cast
- Vomiting or making gurgling noises
- Cannot be awakened from sleep or is unable to speak
- Breathing is very slow or stopped
- Heartbeat is very slow or stopped

Signs of **OVERMEDICATION**, which may progress to overdose, include:

- Unusual sleepiness or drowsiness
- Mental confusion
- Slurred speech
- Intoxicated behavior
- Slow or shallow breathing • Pinpoint pupils
- Slow heartbeat and/or Low blood pressure



- Difficulty waking the person from sleep

If you are being treated for dependence on opioid drugs (either an illicit drug like heroin or a medication prescribed for pain), you may experience the following symptoms of opioid withdrawal after taking naloxone:

- Feeling nervous
- Restless; Irritable
- Body aches
- Dizziness or weakness
- Diarrhea; Stomach pain; Mild nausea
- Fever
- Chills and/or Goosebumps
- Sneezing or runny nose in the absence of a cold

SUMMARY: HOW TO AVOID OPIOID OVERDOSE

- Take medicine only if it has been prescribed to you by your doctor.
- Do not take more medicine or take it more often than instructed.
- A relapse (return to use) after a period of abstinence can significantly increase a person's risk for overdose.
- Tolerance for substances decreases after a period of abstinence.
- Never use alone, there would be no one to assist you in the event of an overdose.
- Dispose of unused medication properly.
- Call a doctor if your pain gets worse.
- Never mix pain medicines with alcohol, sleeping pills, or any illicit substance.
- Learn the signs of overdose and how to use naloxone to avoid a fatal overdose.
- Teach your family and friends how to respond to an overdose.
- Store your medicine in a safe place out of reach of children or pets.

FACTORS THAT CAN INCREASE OVERDOSE RISK

(Please circle any of the risk factors below that might apply to you)

- Past Overdose Events: If you have had a previous overdose, research indicates you may be at much higher risk for future overdoses.



OPIOID OVERDOSE PREVENTION PLAN

- Tolerance: Can change for a variety of reasons: weight gain or loss, starting or stopping medications, depression/exhaustion, use after a period of abstinence
- Health status: Overdose is more likely if the body is physically compromised in any way including such things as: weakened immune system, damaged kidney or liver, dehydration, poor nutrition, recovering from recent illness or surgery, etc.
- Route of Administration: Snorting or smoking drugs has a lower risk of overdose than injection. Any route, if taken in enough quantities can cause overdose.
- Accumulation of too many drugs in the body over time: An overdose can occur because "the high" is over faster than the body can metabolize the drugs. "Re- doses" and/or losing track of drugs taken can cause toxic accumulation.
- Unknown Drug Quality or Potency: Street and internet drugs have no quality control mandates. The potency and effect can vary with each purchase. A person's usual supply/use can lead to overdose if unexpectedly strong.

Client Signature

Date



SELF-HARM AGREEMENT

Client Name: _____

MRN: _____

DOB: _____

I agree to abstain from harming, injuring, and/or endangering myself in any way including attempting suicide while I remain in treatment at **S&B Behavioral Health Care LLC**.

I agree to seek the assistance of a staff member immediately if and when I have any thoughts of self harm and/or harm to others, regardless of the time of day or night.

Client Signature

Date



**AUTHORIZATION FOR INTERNAL DISCLOSURE OF SUBSTANCE USE DISORDER
INFORMATION FOR INSURANCE AUTHORIZATION AND UTILIZATION REVIEW**

Client Name: _____

MRN: _____

DOB: _____

I authorize **S&B Behavioral Health Care LLC**, a federally assisted substance use disorder treatment program, to disclose information from my substance use disorder records to **non-clinical administrative staff within S&B Behavioral Health Care LLC**, specifically the **Authorization and Utilization Management Department**, for the limited purpose of obtaining, managing, and maintaining insurance authorization and payment for my treatment services.

Information to Be Disclosed

This authorization permits access to and disclosure of only the minimum necessary information required to support insurance authorization, continued stay review, concurrent review, appeals, and related payer communications. This may include my substance use disorder diagnosis, level of care, ASAM criteria and determinations, treatment plans, service dates, medical necessity narratives, progress summaries, and other supporting clinical documentation required by Carelon Maryland Behavioral Health or other applicable payers. This authorization does not permit disclosure of psychotherapy notes except as allowed by law.

Purpose of Disclosure

The purpose of this disclosure is insurance authorization, utilization review, care coordination with the payer, and payment determination for covered behavioral health and substance use disorder services.

Expiration

This authorization will expire upon the earliest of the following events: my discharge from treatment with S&B Behavioral Health Care LLC, termination of my insurance coverage applicable to these services, or one year from the date of signature below, unless revoked earlier in writing.



Right to Revoke

I understand that I may revoke this authorization at any time by submitting a written revocation to S&B Behavioral Health Care LLC. I understand that revocation will not apply to information already disclosed in reliance on this authorization and that revocation may impact the program's ability to obtain or continue insurance authorization and payment for my treatment services.

Redisclosure Notice (Required by Federal Law)

This information has been disclosed to you from records protected by federal confidentiality rules (42 CFR Part 2). The federal rules prohibit you from making any further disclosure of this information unless further disclosure is expressly permitted by the written consent of the individual to whom it pertains or as otherwise permitted by 42 CFR Part 2. A general authorization for the release of medical or other information is not sufficient for this purpose.

Acknowledgment

I understand that my substance use disorder information is protected by federal law and that this authorization is voluntary. I have had the opportunity to ask questions and receive answers about this authorization.

Signature of Client or Legal Representative

Date

If signed by Legal Representative, Relationship and Authority: _____





Authorization for Carelon Behavioral Health to Release Confidential Information

Important: By completing all sections of this form you allow Carelon Behavioral Health, Inc. (Carelon Behavioral Health) to disclose health care information to the individuals you identify for up to one year. You may allow Carelon Behavioral Health to share health care information with your family, providers, legal representative, or **anyone** you wish to have access. Please fill in all sections as incomplete forms may be returned.

Please note: It is also important for your doctor to have access to your medical information to ensure you receive the best care possible, including any follow-up care that may be needed. To allow Carelon Behavioral Health the ability to send your health care information to your doctor, complete and sign this form. We will only send information that pertains to your care.

If your request involves alcohol or substance use information, please pay attention to the special instructions in the applicable sections.

SECTION 1: WHOSE HEALTH CARE INFORMATION IS TO BE RELEASED?

I, _____ (Member Name) authorize Carelon Behavioral Health (or any Carelon Behavioral Health subsidiary holding my information) to disclose my health care information as described below.

Additional Member Identifying Information Member ID#: _____ DOB: _____
Phone Number: _____ Name of Health Plan: _____

SECTION 2: WHO IS TO RECEIVE THIS HEALTH CARE INFORMATION?

Print the Name(s) of person, provider or entity who will be receiving your information and contact information (if known):

Dr Bolani Olugboja DNP PMHNP Liberty Family Care 8515 Liberty Road Suite B Randallstown, MD 21133. Office 410-301-6767

Betsy White MSN, APRN, AGPCNP-C, PMHNP-BC Keye Wellness 5411 Old Frederick Rd Unit 18 Catonsville, MD 21229.

Office: (443) 750-0222 Cell: (667) 305-8683

Mcleans Couch Therapeutic Services 4103b Holbrook Rd Randallstown MD 21133. Office: 443-983-8436

Cornerstone Treatment Center Inc 1806 Pennsylvania Avenue Baltimore, MD 21217. Office: 410-630-1090

Phone number of who will be receiving your information: _____

Is it ok to include information from past, present, and/or future treating provider(s)?: ☒ Yes ☐ No

SECTION 3: WHY SHOULD THIS HEALTH CARE INFORMATION BE RELEASED?

Reason ("At my request" is an acceptable response): _____

Specify, if possible: ☒ Care Coordination/Management ☒ Claim Assistance ☒ Quality of Care Review
☐ Other (Please explain reason): _____

SECTION 4: WHAT HEALTH CARE INFORMATION MAY BE RELEASED?

BY INITIALING the items on the following page, you authorize Carelon Behavioral Health to release specific types of information to the party identified in Section 2 above:



Authorization for Carelon Behavioral Health to Release Confidential Information

____ Mental health information and/or records (INITIALS REQUIRED)

____ Alcohol or substance use information and/or records (INITIALS REQUIRED)

Optional: ☒ Claims info ☒ Authorizations ☒ Explanation of benefit letters ☒ Denials/Appeals info ☒ Clinical notes

____ HIV/AIDS related information and/or records (INITIALS REQUIRED)

____ Other health information, please specify (INITIALS REQUIRED): _____

Special instructions, if any (you may specify provider, date span, service type, etc.): _____

SECTION 5: HOW LONG SHOULD THIS AUTHORIZATION LAST?

This authorization shall be in force and effect **for one year** or until I revoke it, in the manner described below or until (insert expiration date or event) _____ discharge _____ (whichever is shorter).

SECTION 6: WHAT ARE MY RIGHTS?

- You have a right to request a copy of this form and to request a copy of the information that is being disclosed.
- You do not have to sign this authorization and your refusal will not affect your benefits unless this authorization is necessary to determine your benefits.
- The information disclosed by this authorization may be at risk for re-disclosure by the recipient and if that happens, it might no longer be protected by federal privacy laws.
- You have a right to revoke this authorization at any time. ***But if you revoke this authorization, the revocation will not affect the disclosure of any information that Carelon Behavioral Health has already sent to the recipient.***
- If you authorized release of alcohol or substance use information to a healthcare organization that is not your treating provider, for the next two years, you have the right to find out who within that organization actually saw your information. You should contact the organization directly for that information.

Please note that if you have authorized the release of ONLY alcohol or substance use treatment records, you may revoke this authorization verbally. Revocation involving all other types of health care records must be in writing.

Signature of the Member or the Member's Legally Authorized Representative*

Date

Print Name

*** NOTE: If you are signing as the individual's Legally Authorized Representative, attach a copy of the appropriate legal document(s) granting you the authority to do so. Examples would be a health care power of attorney, a court order, guardianship papers, etc. A financial or business power of attorney is NOT sufficient.**

Please contact the phone number for behavioral health, mental health, or substance use services on your medical ID card with any questions or to determine where to mail or fax your request.